



ARIZONA HOUSE OF REPRESENTATIVES FLOOR AMENDMENT EXPLANATION

57th Legislature, 2nd Regular Session

Majority Research Staff

SB 1345: health facilities; anonymous complaints; prohibition

GRESS FLOOR AMENDMENT

1. Removes the provision that prohibits DHS from accepting or investigating anonymous complaints against health care institutions unless the complaint is from the subject of the complaint allegation or a witness to the alleged conduct in the complaint.

ADDITIONAL COW
GRESS FLOOR AMENDMENT
HOUSE OF REPRESENTATIVES AMENDMENTS TO S.B. 1345
(Reference to House engrossed Senate bill)

Amendment instruction key:

[GREEN UNDERLINING IN BRACKETS] indicates text added to statute or previously enacted session law.
[Green underlining in brackets] indicates text added to new session law or text restoring existing law.
[GREEN STRIKEOUT IN BRACKETS] indicates new text removed from statute or previously enacted session law.
[Green strikeout in brackets] indicates text removed from existing statute, previously enacted session law or new session law.
<<Green carets>> indicate a section added to the bill.
<<Green strikeout in carets>> indicates a section removed from the bill.

1 The bill as proposed to be amended is reprinted as follows:

2 Section 1. Section 36-405, Arizona Revised Statutes, is amended to
3 read:

4 36-405. Powers and duties of the director

5 A. The director shall adopt rules to establish minimum standards
6 and requirements for constructing, modifying and licensing health care
7 institutions necessary to ensure the public health, safety and welfare.
8 The standards and requirements shall relate to the construction,
9 equipment, sanitation, staffing for medical, nursing and personal care
10 services, and recordkeeping pertaining to administering medical, nursing,
11 behavioral health and personal care services, in accordance with generally
12 accepted practices of health care. The standards shall require that a
13 physician who is licensed pursuant to title 32, chapter 13 or 17 medically
14 discharge patients from surgery and shall allow an outpatient surgical
15 center to require that either an anesthesia provider who is licensed
16 pursuant to title 32, chapter 13, 15 or 17 or a physician who is licensed
17 pursuant to title 32, chapter 13 or 17 remain present on the premises
18 until all patients are discharged from the recovery room. Except as
19 otherwise provided in this subsection, the director shall use the current
20 standards adopted by the joint commission on accreditation of hospitals
21 and the commission on accreditation of the American osteopathic
22 association or those adopted by any recognized accreditation organization
23 approved by the department as guidelines in prescribing minimum standards
24 and requirements under this section.

25 B. The director, by rule, may:

26 1. Classify and subclassify health care institutions according to
27 character, size, range of services provided, medical or dental specialty

1 offered, duration of care and standard of patient care required for the
2 purposes of licensure. Classes of health care institutions may include
3 hospitals, infirmaries, outpatient treatment centers, health screening
4 services centers and residential care facilities. Whenever the director
5 reasonably deems distinctions in rules and standards to be appropriate
6 among different classes or subclasses of health care institutions, the
7 director may make such distinctions.

8 2. Prescribe standards for determining a health care institution's
9 substantial compliance with licensure requirements.

10 3. Prescribe the criteria for the licensure inspection process.

11 4. Prescribe standards for selecting health care-related
12 demonstration projects.

13 5. Establish nonrefundable application, **INITIAL LICENSURE**
14 **INSPECTION** and licensing fees for health care institutions, including a
15 grace period and a fee for the late payment of licensing fees.

16 6. Establish a process for the department to notify a licensee of
17 the licensee's licensing fee due date.

18 7. Establish a process for a licensee to request a different
19 licensing fee due date, including any limits on the number of requests by
20 the licensee.

21 C. The director, by rule, shall adopt licensing provisions that
22 facilitate the colocation and integration of outpatient treatment centers
23 that provide medical, nursing and health-related services with behavioral
24 health services consistent with article 3.1 of this chapter.

25 D. The director shall establish a model in rule for the department
26 to monitor health care institutions on-site that are found to not be in
27 substantial compliance with the applicable licensure requirements. The
28 director shall establish on-site monitoring fees for health care
29 institutions that are subject to the on-site monitoring requirements. The
30 department may not charge a fee pursuant to this subsection for a
31 complaint or compliance-related survey or inspection if a health care
32 institution is in substantial compliance.

33 E. The department may provide in-service training to health care
34 institutions that request in-service training relating to regulatory
35 compliance outside of the survey process. The director shall establish in
36 rule in-service training fees for health care institutions that request
37 in-service training from the department.

38 F. Ninety percent of the fees collected pursuant to this section
39 shall be deposited, pursuant to sections 35-146 and 35-147, in the health
40 services licensing fund established by section 36-414 and ten percent of
41 the fees collected pursuant to this section shall be deposited, pursuant
42 to sections 35-146 and 35-147, in the state general fund.

43 G. Subsection B, paragraph 5 of this section does not apply to a
44 health care institution operated by a state agency pursuant to state or
45 federal law or to adult foster care residential settings.

1 <<Sec. 2. Title 36, chapter 4, article 1, Arizona Revised Statutes,
2 ~~is amended by adding section 36-420.06, to read:~~
3 ~~36-420.06. Anonymous complaints; prohibition; exceptions~~
4 ~~THE DEPARTMENT OF HEALTH SERVICES MAY NOT ACCEPT OR INVESTIGATE AN~~
5 ~~ANONYMOUS COMPLAINT AGAINST A HEALTH CARE INSTITUTION UNLESS THE COMPLAINT~~
6 ~~IS FROM EITHER OF THE FOLLOWING:~~
7 ~~1. THE SUBJECT OF THE ALLEGATION WITHIN THE COMPLAINT.~~
8 ~~2. A WITNESS TO THE CONDUCT BEING ALLEGED IN THE COMPLAINT.>>~~
9 Sec. 2. Section 36-425, Arizona Revised Statutes, is amended to
10 read:
11 36-425. Inspections; issuance of license; posting
12 requirements; provisional license; denial of
13 license
14 A. On receipt of a properly completed application for a health care
15 institution license, the director shall conduct an inspection of the
16 health care institution as prescribed by this chapter. If an application
17 for a license is submitted due to a planned change of ownership, the
18 director shall determine the need for an inspection of the health care
19 institution. Based on the results of the inspection and after the
20 submission of the applicable licensing fee, the director shall either deny
21 the license or issue a regular or provisional license. A license issued by
22 the department shall be posted in a conspicuous location in the reception
23 area of that health care institution.
24 B. The director shall issue a license if the director determines
25 that an applicant and the health care institution for which the license is
26 sought substantially comply with the requirements of this chapter and
27 rules adopted pursuant to this chapter and the applicant agrees to carry
28 out a plan acceptable to the director to eliminate any deficiencies. The
29 director shall not require a health care institution that was designated
30 as a critical access hospital to make any modifications required by this
31 chapter or rules adopted pursuant to this chapter in order to obtain an
32 amended license with the same licensed capacity the health care
33 institution had before it was designated as a critical access hospital if
34 all of the following are true:
35 1. The health care institution has subsequently terminated its
36 critical access hospital designation.
37 2. The licensed capacity of the health care institution does not
38 exceed its licensed capacity before its designation as a critical access
39 hospital.
40 3. The health care institution remains in compliance with the
41 applicable codes and standards that were in effect at the time the
42 facility was originally licensed with the higher licensed capacity.
43 C. A health care institution license does not expire and remains
44 valid unless:
45 1. The department subsequently revokes or suspends the license.
46 2. The license is considered void because the licensee did not pay
47 the licensing fee, civil penalties or provider agreement fees before the

1 relevant due date or did not enter into an agreement with the department
2 before the relevant due date to pay all outstanding fees or civil
3 penalties.

4 D. Except as provided in section 36-424, subsection B and
5 subsection E of this section, the department shall conduct a compliance
6 inspection of a health care institution to determine compliance with this
7 chapter and rules adopted pursuant to this chapter at least once annually.

8 E. If the department determines a facility, except for a
9 residential care institution or a nursing care institution that does not
10 have the same direct owner or indirect owner as a hospital licensed
11 pursuant to this chapter, to be deficiency free on a compliance survey,
12 the department shall not conduct a compliance survey of that facility for
13 twenty-four months after the date of the deficiency free survey. This
14 subsection does not prohibit the department from enforcing licensing
15 requirements as authorized by section 36-424.

16 F. A hospital licensed as a rural general hospital may provide
17 intensive care services.

18 G. The director shall issue a provisional license for a period of
19 not more than one year if an inspection or investigation of a currently
20 licensed health care institution or a health care institution for which an
21 applicant is seeking a license reveals that the health care institution is
22 not in substantial compliance with department licensure requirements and
23 the director believes that the immediate interests of the patients and the
24 general public are best served if the health care institution is given an
25 opportunity to correct deficiencies. The applicant or licensee shall agree
26 to carry out a plan to eliminate deficiencies that is acceptable to the
27 director. The director shall not issue consecutive provisional licenses
28 to a single health care institution. The director shall not issue a
29 license to the current licensee or a successor applicant before the
30 expiration of the provisional license unless the health care institution
31 submits an application for a substantial compliance survey and is found to
32 be in substantial compliance. The director may issue a license only if
33 the director determines that the health care institution is in substantial
34 compliance with the licensure requirements of the department and this
35 chapter. This subsection does not prevent the director from taking action
36 to protect the safety of patients pursuant to section 36-427.

37 H. Subject to the confidentiality requirements of articles 4 and 5
38 of this chapter, title 12, chapter 13, article 7.1 and section 12-2235,
39 the licensee shall keep current department inspection reports at the
40 health care institution. Unless federal law requires otherwise, the
41 licensee shall post in a conspicuous location a notice that identifies the
42 location at that health care institution where the inspection reports are
43 available for review.

44 I. A health care institution shall immediately notify the
45 department in writing when there is a change of the chief administrative
46 officer specified in section 36-422, subsection A, paragraph 1,
47 subdivision (g).

1 J. When the department issues an original license or an original
2 provisional license to a health care institution, it shall notify the
3 owners and lessees of any agricultural land within one-fourth mile of the
4 health care institution. The health care institution shall provide the
5 department with the names and addresses of owners or lessees of
6 agricultural land within one-fourth mile of the proposed health care
7 institution.

8 K. In addition to the grounds for denial of licensure prescribed
9 pursuant to subsection A of this section, the director may deny a license
10 because an applicant or anyone in a business relationship with the
11 applicant, including stockholders and controlling persons, has had a
12 license to operate a health care institution denied, revoked or suspended
13 or a license or certificate issued by a health profession regulatory board
14 pursuant to title 32 or issued by a state agency pursuant to chapter 6,
15 article 7 or chapter 17 of this title denied, revoked or suspended or has
16 a licensing history of recent serious violations occurring in this state
17 or in another state that posed a direct risk to the life, health or safety
18 of patients or residents.

19 L. THE DEPARTMENT SHALL:

20 1. WITHIN NINETY DAYS AFTER RECEIVING A COMPLETE INITIAL
21 APPLICATION FOR A HEALTH CARE INSTITUTION LICENSE, APPROVE OR DENY THE
22 APPLICATION.

23 2. WITHIN THIRTY DAYS AFTER RECEIVING SURVEY RESULTS COMPLETED BY A
24 THIRD-PARTY ACCREDITATION ORGANIZATION, REVIEW AND SUBMIT TO THE MEDICARE
25 ADMINISTRATIVE CONTRACTOR FOR CERTIFICATION SURVEY RESULTS THAT MEET THE
26 REQUIREMENTS OF THE CENTERS FOR MEDICARE AND MEDICAID SERVICES.

27 ~~L.~~ M. In addition to the requirements of this chapter, the
28 director may prescribe by rule other licensure requirements.

29 Enroll and engross to conform

30 Amend title to conform

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